

A decorative graphic on the left side of the slide consists of two overlapping parallelograms. The front one is blue and the back one is a light green. They are positioned diagonally, with the blue one partially covering the green one.

A History of Mistrust Process

Avery Day



Overview and Objective

Overview:

This three-part Instagram campaign uses bold, accessible visual storytelling to explore why marginalized communities—particularly Black, Indigenous, Latinx, and LGBTQ+ individuals—have struggled to trust the American healthcare system. Each post features ten slides combining striking typography, limited color palettes, and powerful historical context to educate and inspire action.

Objective:

To create an honest yet hopeful educational resource that acknowledges historical and ongoing harms in healthcare, while offering realistic ways to rebuild trust and promote equity. The campaign aims to raise awareness and spark conversation among a social media audience through direct, empathetic messaging



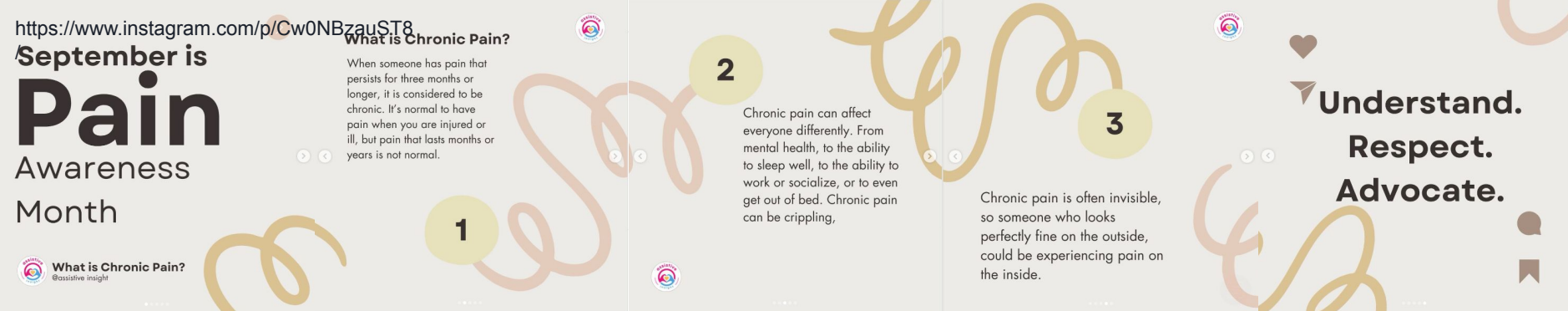
Research and Approach

Each carousel post addresses a key aspect of the issue:

- Post 1 outlines the history of medical abuse and betrayal, including the Tuskegee Syphilis Study, forced sterilization, and harmful psychological practices.
- Post 2 focuses on the ongoing HIV/AIDS epidemic and how systemic racism, stigma, and discrimination continue to affect outcomes for queer and BIPOC communities.
- Post 3 shifts the tone toward actionable hope, offering culturally competent solutions such as community-led clinics, inclusive policy, and provider diversity.

Visual design choices—including flat color fields, high-contrast type, and minimalistic iconography—were intentionally selected to emphasize clarity and emotional impact. Each slide was designed to stand alone, yet build on the last.

Tools Used: Procreate, Canva, Instagram



Mood Board



Sketches

<u>A History of Mistrust:</u> Why Some Communities Struggle to Trust Doctors 1	People of Color (PoC) & LGBTQ+ individuals have faced <u>Systematic Discrimination</u> in healthcare 2	<u>Tuskegee Experiment (1932-1972):</u> 40 years of denying treatment to Black men with Syphilis (and lying to them about it) 3	Black, Indigenous, & Latinx Women were <u>Sterilized without Consent</u> this went on well into the 20th century 4	<u>Queer ≠ Mental Illness</u> homosexuality was classified as a Mental Disorder treated with Shock therapy & Conversion therapy until 1973 5	<u>HIV/AIDS Crisis</u> The government's slow response disproportionately harmed LGBTQ+ & PoC communities 6
Today, many PoC & LGBTQ+ individuals distrust medical providers 7	Mistrust leads to: • late diagnoses • Poorer Mental health • Non-Adherence to treatment • Higher Mortality Rates 8	"Health is more than the absence of disease. Health is about Jobs & employment, education, the environment, & all of these things that go into making us healthy." - Dr. Joycelyn Elders 9	Advocate for Inclusivity training for all healthcare providers lets break the Mistrust 10	<u>AIDS Care:</u> Where we are today 11	Black & Latinx individuals are 3x more likely to contract HIV. <u>Why?</u> 12
Systematic barriers prevent access to lifesaving treatments like <u>PrEP</u> 13	• High costs • Access to healthcare • Stigma & discrimination all contribute to these disparities 14	LGBTQ+ people & PoC often receive inferior treatment & face dismissal of symptoms 15	This discrimination leads to avoidance of <u>crucial treatment</u> 16	This avoidance & mistrust leads to: • later diagnoses • worse health outcomes 17	The AIDS crisis emerged in the early 1980's, claiming the lives of <u>30 million people worldwide</u> 18
<u>Do your part!</u> • Get tested • Destigmatize • Encourage your loved ones to get tested 19	With the right response, we can <u>end the HIV epidemic in America</u> 20	Rebuilding trust between marginalized communities and healthcare providers 21	Restoring trust is critical to closing the gap in medical care for marginalized communities 22	<u>Community-led Clinics</u> LGBTQ+ & PoC-led clinics provide safe, affirming care 23	<u>Representation Matters</u> Patients trust providers who reflect their race, culture, & experiences 24
How do we start? 25	<u>Cultural Competency</u> Training doctors to understand diverse experiences is key to changing how marginalized people receive care. 26	<u>Advocacy & Policy</u> we need policies that require inclusive care & punish discriminatory practices in healthcare systems 27	"We do have the power, if we can come together to make change" - Dr. Karthik Sivasankar 28	<u>Policy changes & community support</u> are needed to ensure inclusive healthcare for all 29	We need inclusive, affordable, accessible, culturally competent healthcare to rebuild trust 30



Sources

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